

U.S. Healthcare Services

U.S. Healthcare Services: A summary of our teach in on specialty pharmacy



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We held a teach-in on Specialty Pharmacy last Wednesday to provide a detailed overview of the specialty pharmacy industry, including its history, economics, and competitive dynamics. This note summarizes key takeaways from the webinar ([see replay here](#) and [slides here](#)).

What is specialty pharmacy - Specialty pharmacy differs from traditional retail pharmacy because it focuses on high-cost, complex therapies, many of which are biologics. Although specialty medications account for only 3-4% of prescription volume, they represent more than half of total drug spending and have been one of the fastest growing areas of the pharmaceutical market. Specialty drugs may be covered under either the pharmacy or medical benefit, with approximately 2/3 of spending occurring through the pharmacy benefit and 1/3 through the medical benefit, depending on how the therapy is administered.

Business models - Unlike PBMs, which manage drug benefits and negotiate rebates, specialty pharmacies generate revenue by dispensing medications and providing clinical support, making them less exposed to regulatory and policy risk. Compared with retail pharmacies, specialty pharmacies are typically centralized distribution operations that emphasize patient support services and are heavily involved with home delivery.

Specialty pharmacy economics - Despite dispensing relatively few prescriptions, specialty pharmacies generate substantial revenue because specialty drugs are extremely expensive. Profitability is driven by factors such as scale, access to limited-distribution drugs, biosimilar adoption, vertical integration, and payer relationships, with the largest players (CVS, Accredo, and Optum) dominating the market, though increasing regulatory scrutiny could potentially lead to PBM and specialty pharmacy separation in the future.

We will continue our Summer Teach In series with a session on Medicaid Managed Care 7/30 at 11am ET ([Click here to register](#)). This session will entail a detailed exploration of the managed Medicaid industry including its history, competitors, policy impacts, and the current and future outlook.

We will have an additional PBM Teach In on 8/20 at 11am ET ([Click here to register](#)). This session will focus on the emerging transparent PBM model and look to the self-insured employer MCO model for examples of how that business operates.

BERNSTEIN TICKER TABLE

Ticker	Rating	20 Jul 2026		Price Target	TTM Rel. Perf.	Adjusted EPS				Adjusted P/E (x)		
		Cur	Closing Price			Cur	2025A	2026E	2027E	2025A	2026E	2027E
AGL (agilon health)	M	USD	122.10	86.00	112.2%	USD	(24.46)	(4.81)	(4.52)	(5.0)	(25.4)	(27.0)
CNC (Centene)	O	USD	65.84	68.00	117.4%	USD	2.08	3.38	4.23	31.6	19.5	15.6
CI (Cigna)	O	USD	283.94	381.00	(22.2)%	USD	29.84	30.49	33.75	9.5	9.3	8.4
CVS (CVS)	O	USD	107.61	106.00	55.5%	USD	6.75	7.37	8.14	16.0	14.6	13.2
ELV (Elevance)	O	USD	382.23	488.00	19.7%	USD	30.30	27.09	30.60	12.6	14.1	12.5
HCA (HCA)	M	USD	370.42	413.00	(15.6)%	USD	28.21	30.44	33.14	13.1	12.2	11.2
HUM (Humana)	O	USD	398.20	425.00	62.5%	USD	16.60	9.12	15.41	24.0	43.6	25.8
MOH (Molina)	O	USD	229.71	286.00	7.3%	USD	11.03	5.24	11.80	20.8	43.8	19.5
UNH (UNH)	O	USD	421.55	512.00	30.9%	USD	16.35	20.11	22.11	25.8	21.0	19.1
SPX			7,443.28									

O - Outperform, M - Market-Perform, U - Underperform, NR - Not Rated, CS - Coverage Suspended

Source: Bloomberg, Bernstein estimates and analysis.

INVESTMENT IMPLICATIONS

We rate AGL Market-Perform with a target price of \$86, CNC Outperform with a target price of \$68, CI Outperform with a target price of \$381, CVS Outperform with a target price of \$106, ELV Outperform with a target price of \$488, HCA Market-Perform with a target price of \$413, HUM Outperform with a target price of \$425, Molina Outperform with a target price of \$286, and UNH Outperform with a target price of \$512.

DETAILS

There are several factors that differentiate specialty pharmacy from traditional retail pharmacy. Specialty medications are more likely to be biologics than small-molecule therapies and typically treat complex, chronic, or rare conditions. These drugs are often high cost, prompting some PBMs to incorporate cost thresholds into their definitions of specialty medications. Although specialty medications account for only 3-4% of prescription volume, they represent more than half of total prescription drug spending.

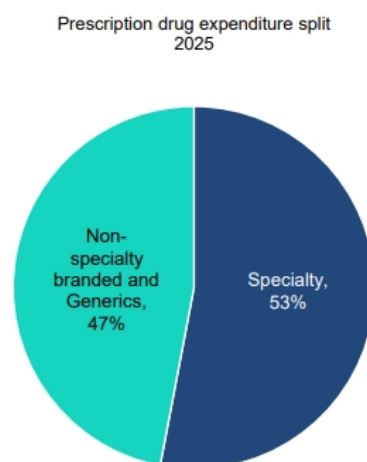
Compared to retail pharmacies, specialty pharmacies allow more control over where drugs are dispensed, how the dispensing process is managed, and the promotion of adherence to make sure the drug is being used as prescribed. In addition, many specialty medications require special handling, storage, and administration.

EXHIBIT 1: What is specialty pharmacy

What is specialty pharmacy

- Multiple ways to define the term
- Biologics vs small molecule
- More complex than traditional orals – handling, who administers, treatment complexity
- PBM definitions including cost

Today specialty represents over half of total drug spend

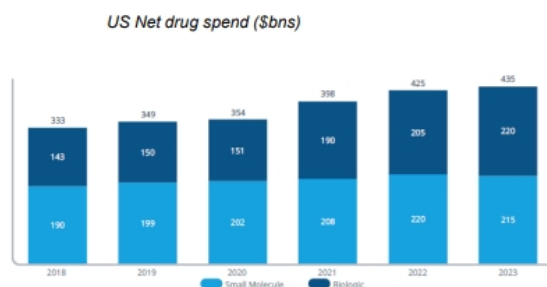
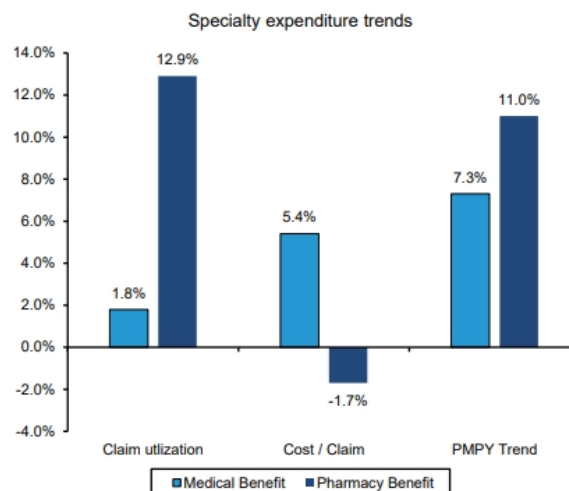


Source: IQVIA, Bernstein analysis

Specialty pharmacy has been one of the fastest-growing segments of the pharmaceutical market. Many newly launched therapies, particularly biologics, enter the market through specialty channels. As a result, specialty drug spending has grown significantly faster than traditional medications, with recent cost trends averaging approximately 9% annually compared with 2.5% for traditional drug therapies.

Specialty medications may be covered under either the pharmacy benefit or the medical benefit, depending on how they are administered. Pharmacy benefit specialty drugs are typically self-administered products, such as oral therapies and self-injectables, while medical benefit specialty drugs are generally administered by a healthcare professional in a physician office, infusion center, or hospital setting.

The drivers of spending differ across these benefit categories. Within the pharmacy benefit, growth is primarily driven by increased utilization as therapies gain broader adoption. Within the medical benefit, spending growth is driven more by increases in unit cost. More recently, the introduction of biosimilars has helped moderate costs and reduce cost per claim in several therapeutic categories. However, specialty drug expenditures continue to rise due to the growing use of these medications.

EXHIBIT 2: **What is specialty pharmacy - growth****Specialty cost trend of 9% vs 2.5% for traditional over recent period****Drivers of growth – mix, price inflation and increased number of prescriptions**

Source: IQVIA, PSG, Bernstein analysis

At an industry level, about 1/3 of specialty drug expenditure is covered by the medical benefit, with the remaining 2/3 being covered by the pharmacy benefit. Because many specialty therapies are administered by healthcare professionals in physician offices, infusion centers, or hospitals, a significant portion of specialty drug spending is covered under the medical benefit rather than the pharmacy benefit.

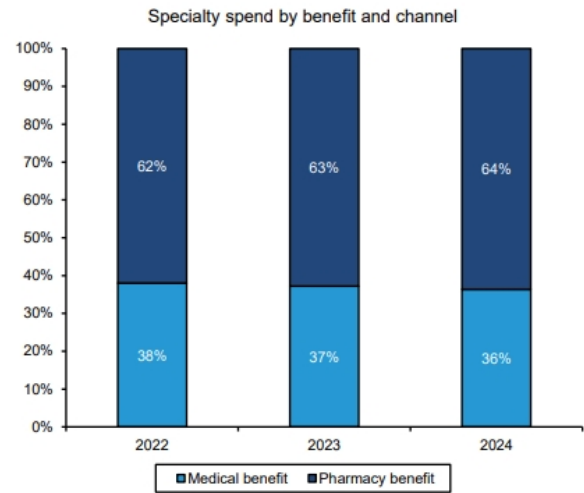
Employers generally have the medical benefit as one contract, and then they get a rider for pharmacy coverage that's administered separately. Items within the medical benefit include doctor visits, hospital stays, and drug administration in a hospital setting. The pharmacy benefit historically is an additional coverage that is added on top of health insurance and covers self-administered medications dispensed through retail, mail-order, or specialty pharmacies.

Unlike traditional retail prescriptions, specialty medications are frequently delivered directly to the patient's home. This distribution model supports enhanced clinical oversight and patient adherence. While some specialty medications are dispensed through retail pharmacies such as CVS or Walgreens, home delivery remains a common distribution channel, particularly for self-administered therapies.

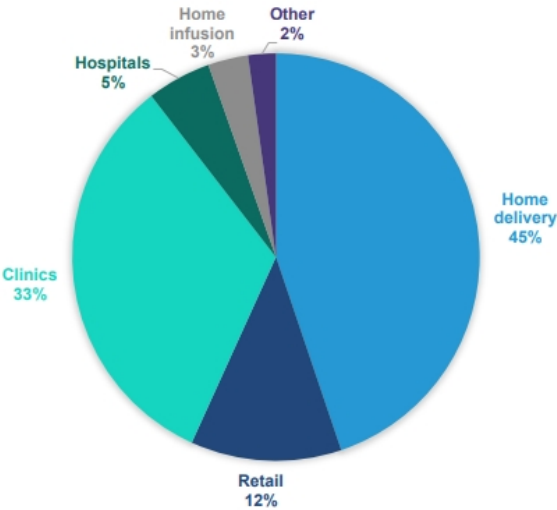
Site of care is an important consideration for specialty medications. Specialty utilization in home infusion settings is particularly high due to the complex administration requirements of many therapies. In [Exhibit 3](#), the reported percentage of administration occurring in hospitals appears lower than what we would expect and may reflect hospital-affiliated outpatient clinics being categorized within the "Clinic" segment rather than the "Hospital" segment.

EXHIBIT 3: **What is specialty pharmacy**

Pharmacy vs medical benefit



Site of administration or purchase



Source: IQVIA, PSG, Bernstein analysis

Specialty pharmacies can generally be grouped into four distinct categories based on the types of therapies and services they provide.

Traditional specialty pharmacy focuses primarily on self-administered therapies, including oral medications and self-injectable drugs. Operations are typically centralized around a big box distribution center with many clinical and support personnel to educate patients and improve adherence.

Rare and orphan follows a similar operating model but is dedicated to therapies for rare diseases and ultra-rare conditions. These medications often serve small patient populations and can carry exceptionally high costs.

Home infusion provides medications that are administered intravenously in a patient's home.

Clinician-administered therapies are delivered by healthcare professionals in physician offices, outpatient clinics, infusion centers, or hospital settings.

EXHIBIT 4: **Specialty pharmacy models**

Traditional Specialty Pharmacy - Orals and self-injectibles	Home infusion
• Scale and efficiency in central fill • Logistics in delivery • Clinical support and specialized condition support • Limited Distribution Deals	• Preparation of medication in sterile conditions • Logistics in delivery of supplies and equipment to home • Nursing services, trainings to patients and caregivers to administer the medication • Monitoring the patients for any side effects • Care coordination between pharmacy, PCP, nurses, home health agencies
Rare and orphan	Clinician administered
• Medication used to treat conditions affecting small patient population • Provides limited distribution network for these drugs • Tailored services for patients with rare conditions • Dedicated expert teams for management of complex conditions and specific therapies • Assistance to patient in navigating challenges associated with rare disease, like insurance coverage, financial assistance, care coordination • Supporting clinical trials for new orphan drugs, providing access to investigational medicines to patients • Limited Distribution Deals	• Administered at doctors' office, infusion centers or hospital • Ensures specific dosages, handling, and administering methods • Coordination with providers, medication management, and adherence to treatment protocols

Source: Bernstein analysis

It is important to distinguish between a specialty pharmacy and a PBM, as they play different roles within the pharmaceutical supply chain. A specialty pharmacy serves patients directly by dispensing medications that have been prescribed by a healthcare provider. Revenue is generated through the dispensing and administration of drugs, but pharmacies must pay the cost to acquire the drug. The pharmacy also has the operating costs of servicing the patient including the cost of the facility and the expertise of the pharmacist, nurses, and support staff. In general, this is not dissimilar from a retail pharmacy.

PBMs function more like managed care organizations. Rather than dispensing medications, they manage prescription drug benefits on behalf of employers and negotiate with drug manufacturers and pharmacies to control overall drug spending. Unlike PBMs, specialty pharmacies do not participate directly in manufacturer rebate arrangements and instead earn revenue primarily through dispensing.

As opposed to a PBM, which is more similar to an insurance business, you do not have rebates flowing through the specialty pharmacy.

With PBMs, one of the questions that commonly gets asked is, "why do you even need a PBM?". The role of a PBM is to try to drive down costs through aggregation, similar to managed care. Specialty pharmacies, by contrast, perform a core healthcare function: dispensing medications and supporting patient care. There is little debate about the need for that role. Consequently, specialty pharmacy businesses have less exposure to policy risk than PBMs, whose business models remain the subject of ongoing regulatory and legislative scrutiny.

EXHIBIT 5: **Specialty pharmacy - basic business model****Pharmacy model**

- Physician prescribes prescription for patient
- Specialty pharmacy is selected for dispensing
- Specialty pharmacy connects with patient and coordinates clinical aspects of treatment, delivery and payment
- Specialty pharmacy charges a \$ amount for the specialty med which equates to a discount to AWP
- Payment to specialty pharmacy is split between PBM and consumer co-pay
- Specialty pharmacy has drug in centralized facility, verifies, pick, pack and ships
- Revenue model – receive payment for drug, cost of drug, dispensing costs of pharmacists and clinical support, rent, shipping, etc

Contrasting with PBM

- Contract with network of pharmacies at discount to AWP
- Negotiate discounts (rebates) with pharma for steerage of prescription volume
- Shifting to a fee-based model, but not taking risk

Source: Bernstein analysis

Retail and specialty pharmacies differ significantly in their customer-facing ("carpeted") and operational ("non-carpeted") functions. On the carpeted side, retail pharmacies operate like traditional storefront businesses. Location is important because they rely on consumer foot traffic and generate revenue from store merchandise, with the pharmacy typically located in the back. Specialty pharmacies generally do not have a retail storefront. They function as dispensing centers, making physical location far less important. Typically, these are recurring medications, so home delivery and using a centralized warehouse are most efficient for this model.

On the non-carpeted side, retail pharmacies are highly automated. Pill counting and bottle filling are often automated, with pharmacists providing final verification before medications are dispensed. Specialty pharmacies may also have automation in their pick, pack, and ship center, but they require a greater degree of personalized patient support. In addition to dispensing medications, specialty pharmacy teams often manage prior authorizations, copay assistance, reimbursement, and billing activities.

Clinical support is also a core component of the specialty pharmacy model. Pharmacists, nurses, and other care team members work closely with patients to ensure they understand their therapy, know how to administer their medication, and remain adherent to treatment. In specialty pharmacies, the pharmacist is in the "non-carpeted" segment, where they can dispense the medication and communicate with the customer from a distance.

EXHIBIT 6: **Dispensing - carpeting vs non-carpeting**

Customer/patient interactions	Dispense pack and ship
• Prescription intake • Payment and insurance including prior auths • Clinical review, verification • Drug interaction review • Clinical guidance • Follow-up	• Medication preparation • Dispensing of medication • Packaging • Shipping / delivery

Source: Bernstein analysis

Customer acquisition in retail and specialty pharmacy differs substantially. In retail pharmacy, customer acquisition is driven by traditional marketing, brand recognition, and convenient store locations. In specialty pharmacy, patient acquisition is influenced less by location and more by payer networks, PBM relationships, manufacturer partnerships, and physician referrals. PBM ownership can be a significant competitive advantage because PBMs may incentivize patients to use affiliated specialty pharmacies through network design and lower out-of-pocket costs. Given the high cost of specialty medications, patients rarely choose out-of-network pharmacies, allowing major PBMs to direct prescription volume toward their own specialty pharmacy businesses.

Specialty pharmacies have limited distribution drugs (LDD) and exclusive distribution drugs (ED). LDDs are dispensed only through a select network of pharmacies, and EDs can only be dispensed through one pharmacy. Access to limited-distribution products can be an important source of competitive advantage.

Another key element is the hub relationship, which refers to the working relationship between a manufacturer-sponsored patient support hub and one or more specialty pharmacies. Patient support hubs assist with benefit verification, prior authorizations, financial assistance, and case management, and often influence which specialty pharmacies receive prescription volume. Physician engagement is also important, as specialty pharmacies seek to build relationships with prescribing providers in order to gain their business.

Scale provides meaningful advantages in specialty pharmacy. Large organizations such as CVS Specialty and Accredo benefit from extensive payer networks, flexibility in plan design, established physician and hub relationships, and greater access to limited-distribution therapies.

Several factors drive success in specialty pharmacy customer acquisition. Strong data and reporting capabilities are increasingly important to provide visibility into patient outcomes and operational performance. Speed-to-fill is another critical metric. Pharmacies that efficiently navigate benefit verification, prior authorization, and financial assistance processes are often more likely to receive future referrals from providers, and hubs could use this metric as a determinant of which specialty pharmacy is going to receive the next prescription. Robust clinical support and the number of pharmacists can also be differentiators. Finally, competitive pricing is another important factor.

EXHIBIT 7: **Customer acquisition**

Major drivers of customer acquisition	Critical success factors for customer acquisition
Access • Plan design / network • Drug availability (LDD, ED) Cost Hub relationship Physician detailing	• Data / reporting capability • Speed to fill • Prior auth and other payment support • Competitive rates / costs • Clinical support and expertise (form of brand equity) • Consumer interface (eg app)

Source: Bernstein analysis

Under the pharmacy benefit, specialty drugs are reimbursed either at a discount to a benchmark price (such as Average Wholesale Price) or at a fixed dollar amount. Pharmacies may also receive separate dispensing fees or administrative fees.

Manufacturer rebates are generally not part of pharmacy reimbursement. Rebates accrue to the PBM, rather than the specialty pharmacy itself. This distinction is particularly relevant for vertically integrated organizations that own both a PBM and a specialty pharmacy. While a specialty pharmacy does not directly earn rebates, an integrated PBM may be able to capture rebate value through formulary management and then incorporate some of that economic benefit into drug sourcing negotiations. As a result, the economic value ultimately realized by an integrated organization may differ from that of an independent pharmacy, even though the rebate itself is not earned at the pharmacy level.

For drugs reimbursed under the medical benefit, reimbursement typically follows a buy-and-bill model, with providers paid based on a benchmark such as Average Sales Price plus a markup, for example ASP + 6%.

Specialty pharmaceuticals are substantially more expensive than traditional retail prescriptions. Even relatively low-cost specialty therapies can carry prices in the thousands of dollars, while many retail prescriptions cost less than \$50. Some specialty products introduced to the market can cost hundreds of thousands of dollars annually per patient. As a result, specialty pharmacies can generate significant revenue from a comparatively small prescription volume.

This can be illustrated by Diplomat Pharmacy's revenue per prescription prior to its acquisition by Optum. As shown in [Exhibit 8](#), the average specialty prescription was approximately \$5,000 compared with roughly \$50 for a traditional retail prescription from CVS.

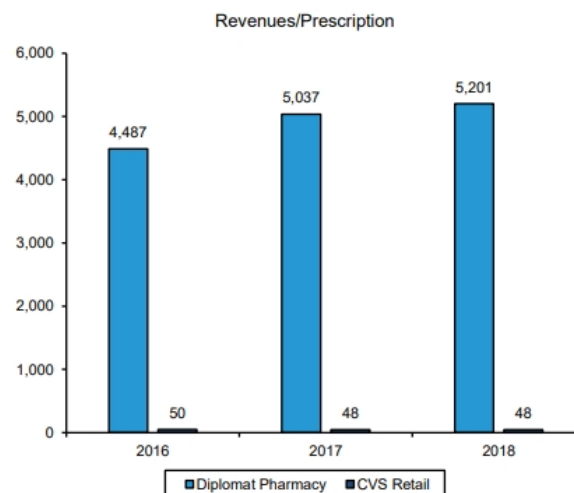
EXHIBIT 8: **Specialty pharmacy pricing****Pharmacy benefit****Pharmacy benefit**

- Discount to AWP or drug specific amount
- Additional dispensing fee
- Data and admin fees from pharma

Rebates would not be in specialty pharmacy, over in a PBM

Medical benefit

- Buy and bill – ASP + 6% or variation

Examples of specialty prices (relatively simple products)

Source: Company reports, Bernstein analysis

Gross margins in specialty pharmacy can vary considerably. Reimbursement economics differ across commercial, Medicare, and Medicaid contracts, and margins can vary significantly between brand and generic products as well as across therapeutic categories. In general, limited-distribution drugs (LDDs) and rare or orphan therapies tend to generate higher margins.

Scale is another important driver of profitability. Larger specialty pharmacies typically benefit from greater purchasing power, stronger relationships with manufacturers, and broader access to biosimilar products. Vertical integration with a PBM can also influence specialty pharmacy economics. Critics, including the FTC, have alleged that vertically integrated PBMs may favor affiliated specialty pharmacies through reimbursement practices, network design, or patient steering.

Vertical integration may also affect pricing and rebate economics. A PBM could offer an employer a guaranteed discount while allocating that discount unevenly across pharmacies in its network. For example, if a PBM promises a 25% discount, it may require an affiliated specialty pharmacy to provide a 23% discount while unaffiliated pharmacies provide a 27% discount, allowing the PBM to achieve the promised aggregate savings while potentially advantaging its own dispensing channel.

Margins also differ by specialty pharmacy type. Traditional specialty pharmacies typically operate with gross margins in the range of approximately 3% to 5%, reflecting greater competition and broader network participation. By contrast, pharmacies focused on limited-distribution, rare-disease, or orphan-drug therapies may earn gross margins of approximately 5% to 7%, benefiting from restricted distribution networks and reduced competitive pressure.

EXHIBIT 9: **Gross margin****Gross margin determinants****Mix is key driver**

- Employer vs government programs
- Brand vs generic
- Therapeutic classes
- Limited or exclusive distribution
- Scale impacts
- PBM Ownership

Traditional Specialty Pharmacy vs LDD / Rare & Orphan Pharmacies*Traditional Specialty Pharmacy*

- Typically 3-5% gross margin
- Biosimilars present great opportunity in this category, with meaningful potential for % margin, but \$ margin impacts are uncertain at this point

LDD and Rare & Orphan

Typically 5-7% gross margin

Potential additional sources of margin

- 3PL – third party logistics provider
- Pharma fee for service / Data and reporting

Source: Bernstein analysis

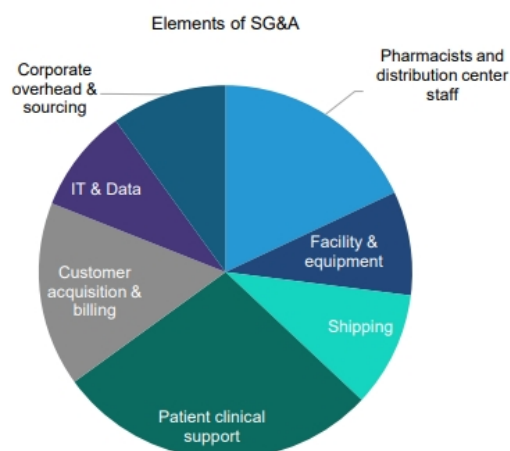
Specialty pharmacies incur a broad range of operating expenses beyond the cost of the medications themselves. As a rough rule of thumb, labor is often the largest expense category, with approximately 1/4 of costs attributable to pharmacists and distribution center staff and another 1/4 attributable to patient clinical support. The remaining expense base is typically spread across shipping and logistics, facilities and equipment, information technology and data infrastructure, customer acquisition, and corporate overhead functions at about 10% each.

Reported margins can also be affected by accounting classifications. Some specialty pharmacies record certain patient-facing service costs within cost of goods, while others report those expenses as operating costs. For example, when Diplomat Pharmacy operated as an independent company, certain nursing and shipping expenses associated with direct patient services were classified within COGS rather than SG&A. Consequently, comparisons of gross margin and operating margin across specialty pharmacies should be made carefully, as differences in accounting presentation can impact reported financials.

EXHIBIT 10: **Key financial levers - bottom line****Operating expenses**

- Clinical / nursing support
- Pharmacists & Distribution ctr staff
- Facilities & equipment
- Shipping and delivery costs
- Customer acquisition and billing
- IT, data and reporting
- Corp overhead

Note: some of these items may be included in cost of goods

Approximation of size of each element

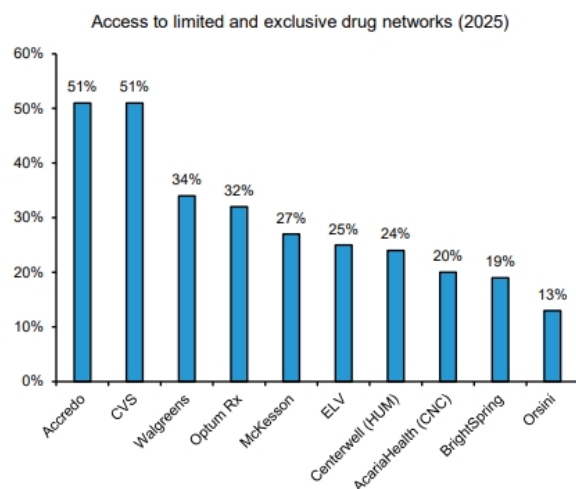
Source: Bernstein estimates, Diplomat Pharmacy company filings

Limited distribution and exclusive distribution drugs have become increasingly common within specialty pharmacy.

These distribution models generally favor either the largest specialty pharmacies, which can offer broad patient access and operational scale, or highly specialized pharmacies with deep expertise in specific disease states. As shown in [Exhibit 11](#), Accredo and CVS each participate in slightly more than half of all limited and exclusive distribution networks. At the same time, certain niche providers have established strong positions within specific therapeutic areas. For example, BrightSpring has developed a significant presence in oncology, while highly specialized pharmacies such as Panther Rare derive most, if not all, of their business from limited distribution drugs.

Limited distribution networks typically include only two to four specialty pharmacies. In many cases, one pharmacy within the network is designated as the third-party logistics (3PL) provider responsible for coordinating inventory management and distribution activities, for which it receives additional compensation.

Drugs distributed through LDD and EDD channels are often among the most clinically complex therapies. These products are frequently concentrated in therapeutic areas such as oncology, immunology, rare/orphan, multiple sclerosis, and other specialty categories that require intensive patient management and clinical support.

EXHIBIT 11: **Limited distribution drugs****LDD size of market and growth****Nature of LDD arrangements****Rationale**

- Specialized handling
- Increased data and reporting needs

Exclusive Distribution

- Single Specialty Pharmacy

Limited Distribution Drugs (LDD)

- Typically 2-4 Specialty Pharmacies
- 3PL needed to manage among pharmacies

Source: Drug Channels Institute, Company filings, Bernstein analysis

Another important consideration in specialty pharmacy is the growth of biosimilars. Unlike limited distribution and exclusive distribution drugs, which are typically branded products, biosimilars create opportunities for specialty pharmacies to improve margins. As patients transition from branded products to lower cost biosimilars, its margin is expected to increase. Dollar margin is expected to remain the same or improve when transitioning from a branded drug to a biosimilar. For example, if a pharmacy earns a \$500 gross profit on a \$10,000 branded drug and later earns the same \$500 profit on a \$5,000 biosimilar, the gross margin percentage effectively doubles.

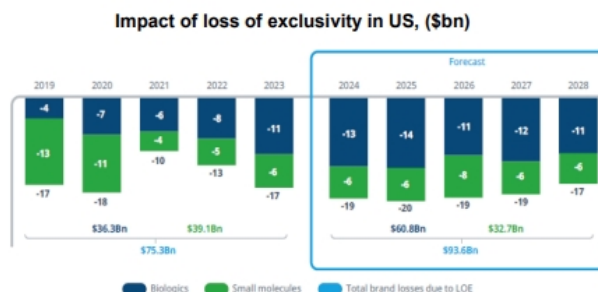
In practice, pharmacies often generate higher dollar margins on generics and biosimilars than on branded products. Our estimates assume dollar-for-dollar profit neutrality when branded drugs are replaced by biosimilars, which may prove conservative and could result in upside for specialty pharmacy earnings.

Biosimilars are frequently launched at discounts of 30% to 50% to AWP, compared with typical branded drug purchasing discounts of approximately 25%. As additional biosimilar competitors enter the market, discounts can become substantially larger, in some cases reaching 80%. As shown in [Exhibit 12](#), loss of exclusivity can result in significant market share loss and pricing pressure for branded drugs.

The benefits of biosimilar adoption may be easier for vertically integrated PBMs to capture given their ability to influence formulary design.

EXHIBIT 12: **Biosimilars****Growing biosimilar opportunity**

- Biosimilar uptake across the addressable market is around 23% as of 2023
- Generic uptake has been fairly robust overall, with generics typically hitting 70% uptake within 12 months
- Manufacturer competition is large driver of the size of discount to brand



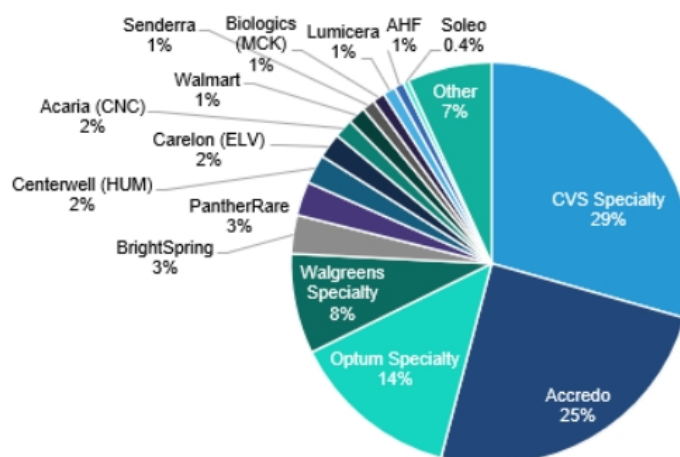
Source: IQVIA, CardinalHealth, Bernstein analysis

CVS and Accredo together account for more than half of the specialty pharmacy market, and including Optum increases the combined share to nearly 70%. Walgreens remains a significant participant as well, although its historical market share may overstate its current position following various divestitures and strategic changes associated with its transition to private ownership.

Looking ahead, we believe there is a meaningful possibility, perhaps in the range of 25% to 35%, that ongoing regulatory and policy scrutiny of vertical integration could ultimately result in the separation of PBMs and specialty pharmacies. In such a scenario, the specialty pharmacies of CVS, Accredo, and Optum could potentially be spunoff. A more fragmented market structure could reduce some of the competitive advantages currently enjoyed by vertically integrated organizations and create opportunities for smaller players to gain share.

EXHIBIT 13: **Major specialty pharmacy players****Major players**

- Big 3 PBMs are largest specialty pharmacies
- Recent growth in next tier of players



Source: Drug Channels Institute, Company filings, Bernstein analysis

Accredo and CVS are the two largest specialty pharmacies in the United States. Accredo differentiates itself through its clinical capabilities, employing approximately 500 specialty pharmacists and more than 600 field nurses. The business also benefits from adjacent assets within the Evernorth platform, including CuraScript in specialty distribution and Shields Health Solutions in the hospital-based and 340B pharmacy market.

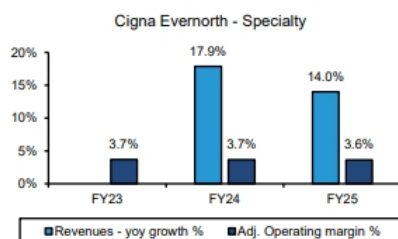
While Evernorth includes businesses beyond specialty pharmacy, Cigna has historically reported operating margins for the segment around 3.5-4%. Given the mix of businesses within Evernorth, a specialty pharmacy gross margin range of approximately 3% to 5% appears reasonable.

CVS Health operates the largest specialty pharmacy platform in the market. Its acquisition of Coram significantly strengthened its home infusion capabilities. Combined with CVS Caremark and the broader CVS Health ecosystem, CVS Specialty benefits from substantial scale, extensive payer relationships, and broad access to specialty drug distribution networks.

EXHIBIT 14: Specialty pharmacy players

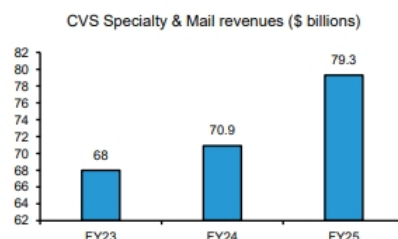
Accredo (CI)

- Specialty pharmacy home delivery and home infusion
- Estimated revenue of >\$60b
- Acquired capabilities: Verity (2019), CarePath Rx (2025); and additional capabilities in related fields such as CuraScript (specialty distributor) and Shields investment (enables hospital/provider specialty pharmacies and 340B)
- 15 Therapeutic Resource Centers with 500 specialty pharmacists, along with 600+ field nurses
- 20+ exclusive products, 80+ LDD products



CVS Specialty Pharmacy

- Specialty pharmacy home delivery, home infusion, and physical pharmacy stores and infusion suites
- Mail and specialty pharmacy revenue of \$79b in 2025
- 1.8m members and 13.5m prescriptions annually
- 35 Centers of Excellence
- Acquired capabilities: Coram (2014), Omnicare (2015)
-



Source: Company reports, Bernstein analysis and estimates

Carelon has been steadily expanding its specialty pharmacy platform through acquisitions, including BioPlus, Kroger Specialty Pharmacy, and Paragon Healthcare. These transactions have broadened the company's capabilities across specialty dispensing and infusion services. As Carelon continues integrating these assets and leveraging Elevance Health's payer relationships, we believe the specialty pharmacy business could ultimately scale to approximately \$10-20b in annual revenue.

OptumRx is the third-largest specialty pharmacy provider in the market. The company has built its position through a series of acquisitions, including BriovaRx, Genoa Healthcare, and Diplomat Pharmacy. Briova strengthened Optum's specialty pharmacy capabilities, Genoa expanded its presence in behavioral health through a network of community-based pharmacy locations, and Diplomat added substantial scale in specialty dispensing.

EXHIBIT 15: **Specialty pharmacy players****Carelon Specialty Pharmacy (ELV)**

- Specialty pharmacy home delivery and home infusion
- Estimated revenue of \$6b
- Acquired capabilities – BioPlus (2023), Kroger Specialty (2024), Paragon Infusion (2024)
- Elevance intends to shift their specialty pharmacy volume to Carelon Specialty Pharmacy once it is capable of handling that amount of business – which could represent an additional \$10-20b depending upon leakage

OptumRx Specialty Pharmacy (UNH)

- Specialty pharmacy home delivery, infusion and physical specialty locations
- Estimated revenue of \$40b
- Capabilities acquired: Brivo (from Catamaran) (2015); Genoa (2018); Diplomat (2020)

Source: Company reports, Bernstein analysis and estimates

Humana's specialty pharmacy business has largely been built organically rather than through large-scale acquisitions. As a result, it remains smaller than the specialty pharmacy platforms operated by CVS, Accredo, and Optum. It seemingly has a strong clinical team for its size, with 600 pharmacists.

Looking ahead, an interesting strategic question is whether Walgreens' new owners ultimately choose to monetize parts of the company's specialty pharmacy operations. Potential buyers could include large integrated healthcare companies such as ELV, UNH, or HUM. The key consideration would be the extent to which Walgreens' specialty pharmacy assets are integrated with its physical retail locations, but we believe the centralized specialty pharmacy business would be readily sellable.

EXHIBIT 16: **Specialty pharmacy players****CenterWell Specialty Pharmacy (HUM)**

- Specialty pharmacy home delivery and home infusion, retail pharmacies within CenterWell primary care clinics
- CenterWell Pharmacy Solutions revenue \$13.0b in 2025
- Acquired capabilities: Enlara Healthcare (2020)
- >600 pharmacists

Walgreens Specialty Pharmacy

- Specialty pharmacy home delivery, home infusion, and physical pharmacy stores and infusion suites
- Estimated revenues were over \$20b, although recent restructuring may have decreased this amount
- >260 specialty pharmacy locations

Source: Company reports, Bernstein analysis and estimates

BrightSpring has established a strong position in oncology-focused specialty pharmacy and has delivered significant growth in recent years, with revenue growth generally in the 20%-30% range. Given its focus on highly specialized therapies, BrightSpring's gross margins tend to be higher than those of more traditional specialty pharmacies and have exceeded 8% in recent years.

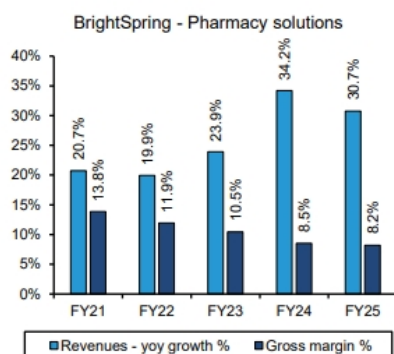
PANTHERx Rare is one of the leading rare-disease specialty pharmacies and has recently attracted attention as a potential PE acquisition target. The company focuses exclusively on rare and orphan therapies, resulting in highly specialized clinical and operational capabilities. Panther is also a frequent participant in limited-distribution drug networks, reflecting its reputation within the rare-disease market. While the company's financial disclosures are limited, we estimate annual revenue of approximately \$1 billion.

EXHIBIT 17: **Specialty pharmacy players****BrightSpring Pharmacy Solutions**

- Primarily specialty home infusion focused, but with capabilities for home delivery and infusion suites
- Infusion and Specialty Pharmacy revenue \$9.1b in 2025
- Acquired capabilities: Onco360 and Amerita through the acquisition of PharMerica (2019)
- Primary focus is on oncology

PANTHERx Rare Pharmacy

- Focused exclusively on rare diseases
- Specialty pharmacy home delivery and distribution
- Frequently the exclusive pharmacy for a drug
- Estimated revenue ~\$1b



Source: Company reports, Bernstein analysis and estimates

The 340B program has grown significantly in recent years and has become an increasingly important component of the pharmacy landscape. 340B is a program which allows eligible hospitals and health care organizations to purchase drugs at discounted prices. These entities purchase eligible drugs at a discount, but are reimbursed at standard rates, allowing them to retain the spread.

While retail pharmacies have historically had the greatest exposure to 340B through contract pharmacy arrangements, specialty pharmacies also participate indirectly through relationships with hospitals, health systems, and other covered entities. As a result, companies with strong hospital and health system pharmacy platforms are likely to see increasing exposure to the 340B market over time. In particular, OptumRx and Express Scripts (with their Shields Health Solutions investment) will have increasing exposure to 340B.

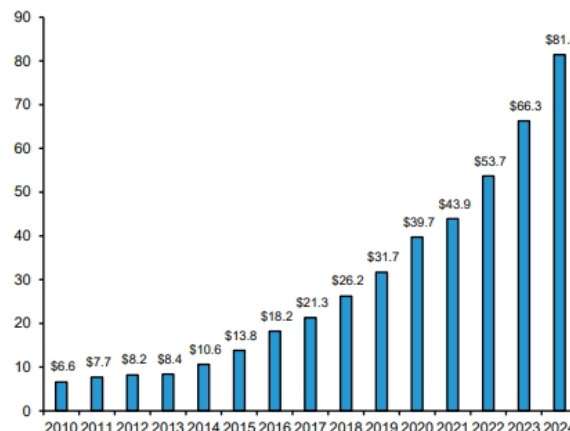
The 340B program has been a target of potential policy change due to its rapid growth. For CY 2027, CMS proposed reducing reimbursement for 340B-acquired drugs to ASP - 33.4%, which would be substantially below the current ASP + 6% methodology.

EXHIBIT 18: **340B program****Background on 340B**

- 340B requires manufacturers in Medicaid and Medicare Part B to sell eligible outpatient drugs at discounted prices to qualifying safety-net entities, so those entities can stretch scarce resources and expand services.
- Eligible participants include six categories of nonprofit or public hospitals plus federal grantees such as FQHCs, Ryan White clinics, and other specified safety-net providers.
- Covered entities generally buy drugs at the 340B price and are reimbursed by insurers at standard rates, retaining the spread. Medicaid fee-for-service is an exception for many retail drugs.
- Covered entities can dispense through in-house pharmacies or contract pharmacies, including specialty pharmacies, to reach patients and capture 340B discounts on eligible outpatient prescriptions.

The program is under scrutiny in part due to rapid growth

Spending on Drugs Purchased Through the 340B Program (2010-2024), \$ billions



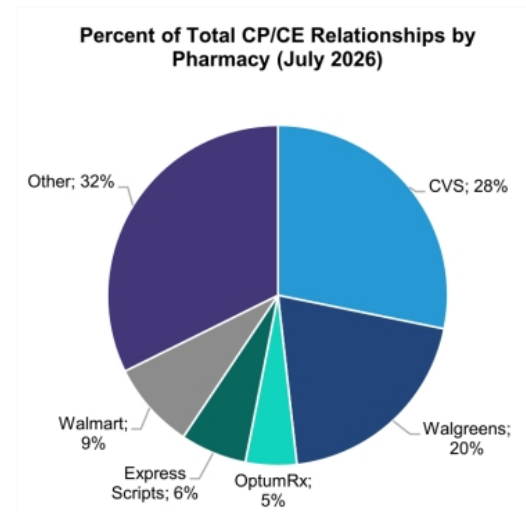
Source: CBO, Bernstein analysis

While 340B is unlikely to be a major long-term growth driver for specialty pharmacy on its own, it remains an important capability for pharmacies serving hospital based and clinician administered therapies. Success in these markets often depends on building strong relationships with hospitals and health systems, including helping them manage specialty pharmacy operations, supporting 340B programs, and providing administrative and clinical services. The ability to effectively partner with hospitals may become an important competitive differentiator.

The number of pharmacies contracting with 340B-enrolled hospitals has more than doubled in the past five years. As can be seen in [Exhibit 19](#), CVS has the highest market share of contracts with covered entities (32% of the market), followed by Walgreens (20%). Their extensive retail footprints and broad geographic coverage likely contribute to their leading positions, making them attractive partners for hospitals and other covered entities.

EXHIBIT 19: **The role of specialty pharmacies in 340B**

- 340B-enrolled hospitals and grantees may elect to dispense 340B drugs to their patients through in-house pharmacies or via contractual arrangements with contract pharmacies.
- As of 2021, there were over 145,000 registered contract pharmacies in HRSA's Office of Pharmacy Affairs Information System database.
- As of July 2026, there are over 330,000 registered participating contract pharmacies as per HRSA's Office of Pharmacy Affairs database



Source: Office of Pharmacy Affairs, Bernstein analysis

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